

Letters to the Editor

Treatments for Social Anxiety Disorder: Considerations Regarding Psychodynamic Therapy Findings

TO THE EDITOR: The article by Falk Leichsenring, D.Sc., et al. (1), published in the October 2014 issue of the *Journal*, offers a useful addition to the literature on the treatment of social anxiety disorder; however, we must emphasize that clinical guidance should not be based on one trial. It should utilize systematic reviews of all available evidence. For example, a recent network meta-analysis including 101 trials found individual cognitive-behavioral therapy (CBT) to be superior to no treatment and both pill and psychological placebos (2). In contrast, psychodynamic therapy was superior to no treatment but not different from either placebo and inferior to CBT (2).

Regarding the finding by Leichsenring et al. that CBT and psychodynamic therapy showed no difference in outcome in the long-term, we are concerned that there were methodologic limitations that detract from these findings. The authors' study registration indicates that the primary outcome measure was the absence of a diagnosis of social anxiety disorder determined by the Structured Clinical Interview for DSM-IV Axis I Disorders, as well as the Liebowitz Social Anxiety Scale. Yet, the authors reported outcomes based on a remission defined by only change in score on the Liebowitz Social Anxiety Scale. We have additional concerns regarding the loss of participants in the long-term follow-up of 81% and 67% in the psychodynamic therapy and CBT groups, respectively. Finally, the use of multiple imputations in an "intention-to-treat" analysis assumes that data are missing at random, but the missing data rate during follow-up was significantly higher for psychodynamic therapy than CBT.

REFERENCES

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2. Mayo-Wilson E, Dias S, Mavranzouli I, et al: Psychological and pharmacological interventions for social anxiety disorder in adults: a systematic review and network meta-analysis. *Lancet Psychiatry* 2014; 1:368–376

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Response to Hofmann et al.

TO THE EDITOR: Hofmann et al. express concerns regarding our study on long-term effects of cognitive-behavioral therapy (CBT) and psychodynamic therapy in social anxiety disorder. Hofmann et al. cite a recent "network meta-analysis" by Mayo-Wilson et al. (1) on the treatment of social anxiety disorder. Drs. Mayo-Wilson and Clark are among the authors of both the meta-analysis and the letter by Hofmann et al. However, this meta-analysis has several severe limitations. Firstly, with regard to psychodynamic therapy, only three studies were included (2–4). In the first study, psychodynamic therapy was **superior** to credible placebo in the treatment of social anxiety disorder (2). Mayo-Wilson et al. found the effects of psychodynamic therapy to be similar to that of psychological placebo (1). This is of note because the study conducted by Knijnik et al. (2) was the only study that directly compared psychodynamic therapy to psychological placebo. Our study is discussed below (4). In the third study included by Mayo-Wilson et al., avoidant personality disorder was examined, not social anxiety disorder (3). Mayo-Wilson et al. did not include a recent randomized controlled trial that found CBT and psychodynamic therapy to be equally efficacious in social anxiety disorder (5). A recent and more comprehensive meta-analysis reported psychodynamic therapy to be as efficacious in anxiety disorders as other bona fide treatments (6). In contrast to Mayo-Wilson et al., our study on social anxiety disorder is one of the few studies in which cognitive-behavioral and psychodynamic researchers collaborated on an equal basis